

Improving pigmentation problems

In patients with acne, particularly those with darker skin phototypes (Fitzpatrick IV–VI), post-inflammatory hyperpigmentation (PIH) is a major concern. Topical retinoids should be the first-line treatment to control both acne and PIH. Adjunctive use of dermocosmetics containing active ingredients—combined with daily sun protection—can help prevent and treat PIH.

The Delphi consensus process guided wording choices based on strength of evidence and expert opinion. In this context, "can help prevent" was ranked lower than stronger recommendations such as "should be recommended" or "can be recommended," reflecting limited but supportive data. However, clinical experience supports the use of dermocosmetics for PIH prevention, especially in high-risk individuals.

Evidence shows benefits including greater reductions in inflammatory and non-inflammatory acne lesions, reduced frequency of acne flares, sustained improvement after discontinuation of medical therapy, and trends toward better global assessment scores.[44,45,48,49,57–59]

An open-label study by DuBois et al. involving 50 patients with Fitzpatrick skin types IV–VI found that adding an oil-control skincare regimen (foam wash and moisturizer with SPF 30) improved quality of life, with the proportion of patients reporting acne as having “no effect” increasing from 16% at baseline to 55% at Week 16.[55] In a randomized, split-face study, Wanitphakdeedecha et al. reported a significant reduction in melanin index in patients using a moisturizer combined with four sessions of photodynamic therapy (PDT).[59]

These findings, along with clinical experience, support the use of dermocosmetics to help control and reduce PIH in acne patients at risk for hyperpigmentation. This may be especially beneficial for patients prone to excoriation, a significant modifiable risk factor for PIH.[60]

DISCUSSION

Dermocosmetics can and should be integrated into comprehensive acne management. A growing body of evidence indicates that dermocosmetics positively impact acne outcomes. This article provides expert recommendations for using dermocosmetics as adjuncts to medical therapy, with Figure 2 offering a practical clinical algorithm for their integration.

Dermocosmetics appear useful as monotherapy in mild acne, with evidence supporting improvements in global assessment, lesion count reduction, tolerability, sebum control, skin barrier function, and maintenance of skin clearance after stopping acne medication. These results are consistent with panel members' clinical observations.

As adjunctive therapy, dermocosmetics enhance treatment efficacy through additive effects and improved tolerability of active medications. These recommendations may also guide other healthcare providers managing mild acne.

Pigmentation concerns are highly significant for a large subset of acne patients, particularly those with darker skin tones. Addressing PIH early with appropriate treatments and preventive strategies, including sun protection and targeted dermocosmetic use, is essential for optimal patient outcomes.